

Trauma-Focused Recovery Model

TFR

What I wish I had understood sooner,
and what I hope recovery can make room for.

Austan Sloan

A personal perspective, informed by research.

01 / WHAT THIS IS

Where this comes from

This model was born at the tail end of a 24-year battle with addiction.

For most of that time, I was taught that my problem was alcohol. Abstain. Build structure. Work the program. Manage cravings. Stay accountable. So I tried. Repeatedly.

I learned about addiction. I practised coping skills. I leaned into the approaches I was offered. What I did not understand was how much of my own struggle was connected to what had happened long before I started drinking.

Learning about trauma gave me a different way to understand that history. I began to see how using had become a way of coping with pain I had never really understood. That is how I make sense of my experience. Other people will have different stories.

I'm writing this as someone who lived through addiction and went looking for answers. I'm not a clinician or a researcher. What I can speak to is what I experienced, what I found useful, and the questions that experience left me with.

The idea behind TFR

The Trauma-Focused Recovery Model brings together a few things I wish had been more connected in my own recovery: getting stable, understanding trauma, knowing what help exists, having a choice about that help, and building a life beyond addiction.

These ideas already appear in trauma and addiction care. Judith Herman's work on safety, processing, and reconnection helped me put them into a clearer picture. TFR is my way of drawing those connections, with particular attention to education and what happens when treatment ends. [1]

Where the research fits

The references support parts of this picture. TFR itself has not been tested as a complete model. I am putting forward an idea for discussion, not instructions for how someone should be treated.

02 / WHY THIS MATTERS

Where I see the gap

In recovery spaces, I kept hearing painful stories. Stories about addiction, certainly, but also about what life had been like before it. Early loss. Unsafe homes. Relationships that taught people to stay on guard.

Those conversations stayed with me. They don't tell me what is true for everyone with addiction. They do help explain why I started asking whether understanding trauma should have a more visible place in recovery.

The research gives that question a foundation. The Adverse Childhood Experiences (ACE) Study found links between childhood adversity and later alcohol and drug problems. That does not mean adversity causes every addiction, or that an ACE score tells someone what treatment they need. It does mean those experiences deserve attention. [3]

Getting stable mattered

Treatment gave me structure and temporary stability. I do not want to lose sight of that. Safety, routine, community, and support can be lifesaving. They helped me.

The difficulty came when I still did not understand what was happening underneath my drinking, or what kind of help might address it. Being more stable did not answer all the questions I was carrying.

A person can leave a program sleeping better, eating better, and feeling hopeful, yet still be unsure where to turn when old distress returns. If trauma is part of that picture, a relapse-prevention plan may need to sit alongside help for those symptoms.

Repeated treatment and returns to use are well documented, but trauma is not the only possible explanation. Health, housing, relationships, dependence, and access to support can all matter. The research on those cycles does not prove that TFR would prevent them. [15, 19]

The question I keep coming back to

When someone needs more help than a program can offer, do they leave knowing what that help might be and how to reach it?

03 / TWO TERMS WORTH UNDERSTANDING

Trauma-informed and trauma-focused

I found this distinction useful because the words can sound as though they mean the same thing.

Trauma-informed care

This describes an approach to supporting people that takes trauma into account: making room for safety, trust, choice, and collaboration, and trying to avoid causing further harm. It can shape the way an entire service works. [2]

Trauma-focused treatment

This is treatment that works directly with trauma-related difficulties. It involves someone trained to help with those problems, using an approach suited to the person and what they are experiencing.

A program can do both. Calling a service trauma-informed does not tell us, by itself, whether it offers trauma therapy. The useful question is what support is actually available, either there or through another service.

Trauma and PTSD are not interchangeable

Post-traumatic stress disorder (PTSD) is a particular condition that needs proper assessment. Someone can have experienced adversity or be struggling with its effects without having PTSD. A lot of the treatment research cited here concerns people with PTSD, so it cannot answer every question about every kind of distress.

That research does show why asking about trauma-related symptoms matters. In one Danish study, 1,347 adults entered the study and 1,286 had enough data for PTSD classification. Of that group, 23.9% screened positive for probable PTSD and 14.5% for symptoms below the full threshold. Those are screening results from five Danish services, not a figure for Alberta or a diagnosis for any individual reader. [5]

An earlier study also found that PTSD was often missed in the addiction-treatment group it examined. That is a reason to make assessment available, rather than assume that someone does or does not have it. [4]

04 / STAGE 1.5

The missing middle: education

This is the part of TFR I feel most strongly about. I call it **Stage 1.5: Trauma Literacy**. In ordinary language, it means giving people a chance to understand what trauma can look like, how it may connect to their struggles, and what help is available.

For a long time, I did not have that understanding. I had explanations for my drinking, but very little language for the experiences underneath it. Learning more helped me make sense of things I had taken as evidence that something was wrong with me.

Some programs already teach this. I would like it to be easier to find, easier to understand, and connected to something people can actually do next.

What I would want people to have a chance to learn

- What trauma can include, while recognizing that people describe their experiences differently.
- How stress and trauma can affect feelings, reactions, and relationships. [20]
- How alcohol or drugs can become a way of seeking relief for some people. [16]
- What trauma treatment involves, what the options are, and what questions to ask.
- Where to find further support, if and when they want it.

No one should have to tell their story to take part. Education can happen during treatment and continue afterwards. It should not become another course someone has to pass before they can get help.

Understanding mattered to me, but I cannot promise it will make shame disappear or make someone ready for therapy. Research links shame, trauma, and substance use; that is different from proving that a particular lesson changes them. A trial of group education found no clear advantage on its main PTSD and behaviour measures. [11, 12]

Even learning about trauma can bring up difficult feelings. People need the option to pause and someone they can turn to for support.

05 / STAGE 2

The next step, and the choice to pause

Understanding something and getting help with it are different steps. For someone still struggling with trauma-related symptoms, therapy may be part of what comes next.

By trauma processing, I mean working with a trained professional on the effects of traumatic experiences. I do not mean asking people to disclose painful memories in a group before they want to, or expecting them to work through those memories alone.

The choice of treatment belongs with the person and their therapist

For PTSD, prolonged exposure (PE), cognitive processing therapy (CPT), and eye movement desensitization and reprocessing (EMDR) have strong research support. Other approaches people may encounter include ART, IFS, and somatic therapies. Their evidence differs, so I would not put them all on the same footing. [13]

For example, a small 2026 study of an IFS-based program found improvement in both that program and the comparison group, without a clear difference in PTSD improvement between them. That is useful research, but it leaves questions open. Research involving both PTSD and substance use is also still developing for that approach. [17, 18]

There is room for different timing

Research supports treating PTSD alongside substance-use problems. A person does not automatically have to finish addiction treatment first. Some people benefit from preparation; others can begin trauma-focused therapy without a separate preparation phase. The decision needs to fit their circumstances and be made with someone qualified to help. [6-10, 14]

That is why the stages in TFR should not become a fixed timetable. They are a way to explain the pieces, not a rule that everyone must move through them in order.

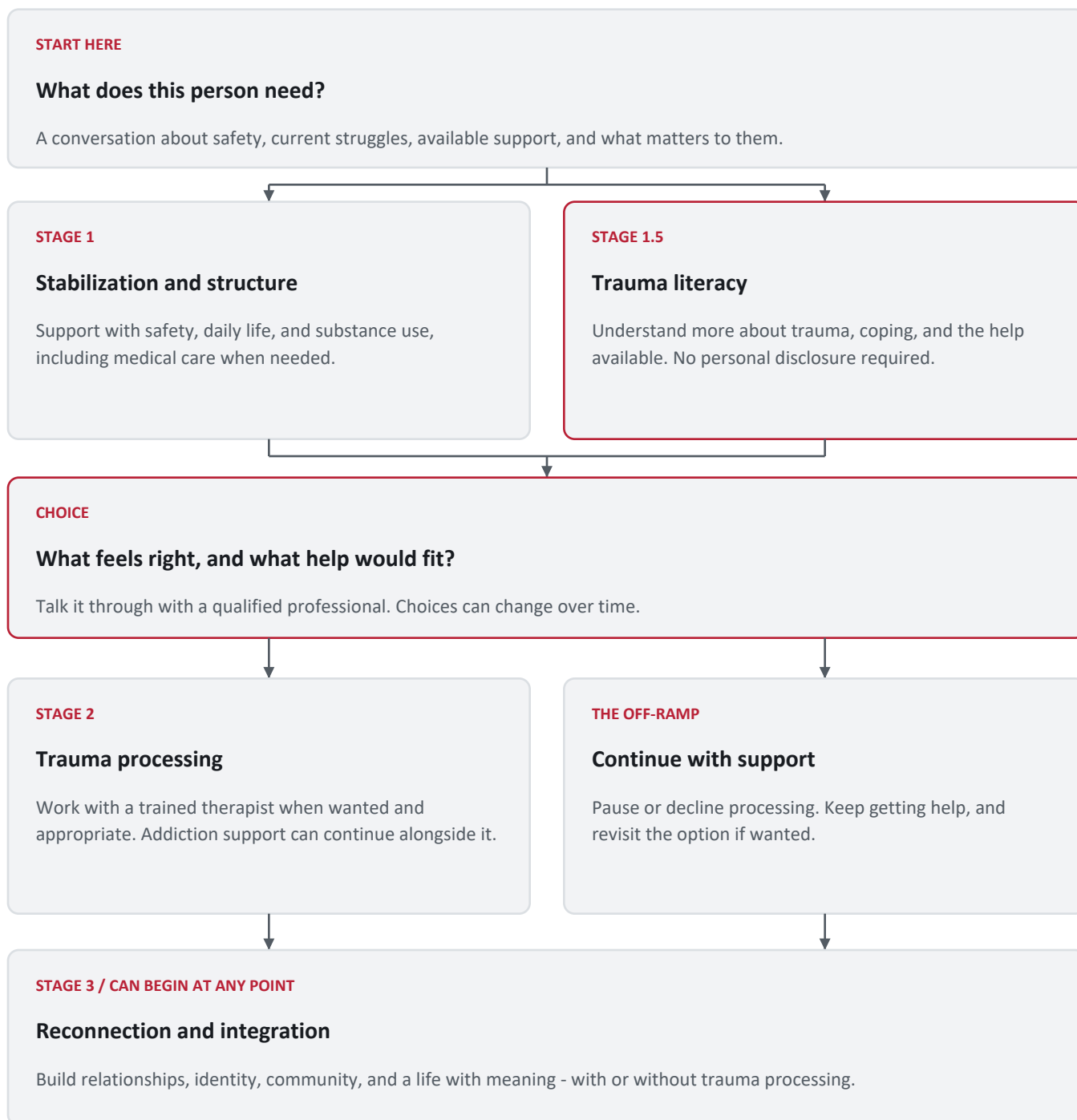
The off-ramp stays

People can pause, decline trauma processing, or return to the idea later. They should still have support and room to build a meaningful life. The door to further help should remain open.

06 / THE TFR MODEL

How the pieces fit

The stages can overlap. Education and rebuilding life can begin early.



This picture draws on Herman's recovery stages and research on trauma and addiction care. It explains the idea; it is not a treatment plan or a requirement to complete each box. [1, 6, 10, 14]

07 / PUTTING THE IDEA INTO PRACTICE

What this could look like

I do not expect every treatment centre to offer every kind of therapy. A centre that cannot provide trauma treatment may still be able to help someone find it.

The part I am asking people to consider is what happens before a person walks out the door. If further help is needed, can the conversation become more specific than “find a therapist”?

Help someone understand the options

Explain what kinds of support might fit, who provides them, and how to ask for an appointment. Make space for questions. A long list of names may be difficult to use when someone is already overwhelmed.

Be honest about access

Cost matters. So do waiting lists, transport, eligibility, and whether a service is taking new people. Public services, community organizations, and insurance-funded options will not be equally available to everyone. A referral needs to take those differences seriously.

Make the next step clear

With the person's permission, help make contact where possible. Before discharge, they should know who they are trying to reach, what they need to do next, and who can help if it falls through.

Leave room for support while waiting

A referral does not remove the need for ongoing addiction care, medication where appropriate, practical help, or connection. People still need support while they decide what to do or wait for an appointment.

I know this takes time and resources. I cannot claim it would cost nothing, fit every program, or prevent another return to treatment. People working in these services will know much more than I do about what is possible in their setting.

The change I am asking for

Make understanding and the next step part of the conversation before treatment ends. Help people leave knowing more about themselves and where they can turn.

08 / WHY I AM PUTTING THIS FORWARD

What I wish had been different

When I first entered treatment, I believed I simply liked using. I had no useful framework for understanding developmental trauma or the way earlier experiences might still be affecting me.

I learned how to manage addiction before I understood much about what I had been trying to manage through it. Looking back, I wish those conversations had happened closer together.

I wish I had been given words for what I was experiencing. I wish someone had explained the kinds of help available and helped me work out how to reach them. I wish understanding myself had been a more visible part of getting better.

I cannot know how that would have changed the course of my recovery. I can say that finding this information mattered to me, and that having to piece it together on my own was part of what led me to build Recover-You.

There is more to recovery than managing a problem

The final part of TFR is reconnection and integration. To me, that means making room for relationships, identity, purpose, and ordinary life. Who am I beyond addiction? What matters to me? What kind of life am I trying to build?

That work can begin at any point. It belongs to people who choose trauma therapy and to people who do not. No one should have to complete a particular kind of treatment before their life is allowed to become bigger.

My hope is that TFR makes a useful conversation easier to have: about what treatment helped with, what someone still needs, and how those needs might be connected to support.

If you work in treatment, you may already be doing much of this. If you are in recovery, some of it may fit your experience and some may not. There is room for both.

What I want this model to hold onto

Stability matters. Understanding matters. Choice matters. And when someone needs further help, knowing where to turn matters too.

Austan Sloan
Recover-You

RESEARCH AND REFERENCES

References 01-07

These are the sources behind the numbered references in the text.

01. Herman, J. L. (1992). *Trauma and Recovery: The Aftermath of Violence - From Domestic Abuse to Political Terror*. Basic Books.

www.hachettebookgroup.com/titles/judith-l-herman-md/trauma-and-recovery/9780465098736/

02. Substance Abuse and Mental Health Services Administration. (2014). *SAMHSA's Concept of Trauma and Guidance for a Trauma-Informed Approach*. HHS Publication No. SMA 14-4884.

library.samhsa.gov/sites/default/files/sma14-4884.pdf

03. Felitti, V. J., et al. (1998). *Relationship of childhood abuse and household dysfunction to many of the leading causes of death in adults: The Adverse Childhood Experiences (ACE) Study*. American Journal of Preventive Medicine, 14(4), 245-258.

[doi.org/10.1016/S0749-3797\(98\)00017-8](https://doi.org/10.1016/S0749-3797(98)00017-8)

04. Gielen, N., Havermans, R. C., Tekelenburg, M., & Jansen, A. (2012). *Prevalence of post-traumatic stress disorder among patients with substance use disorder: It is higher than clinicians think it is*. European Journal of Psychotraumatology, 3, 17734.

doi.org/10.3402/ejpt.v3i0.17734

05. Karsberg, S., Najavits, L., Pedersen, M. U., Elklit, A., & Vang, M. L. (2025). *Trauma and ICD-11 PTSD in substance use disorder treatment: A Danish multi-site study*. BMC Psychiatry, 25, 770.

doi.org/10.1186/s12888-025-07164-w

06. Mills, K. L., et al. (2012). *Integrated exposure-based therapy for co-occurring posttraumatic stress disorder and substance dependence: A randomized controlled trial*. JAMA, 308(7), 690-699.

doi.org/10.1001/jama.2012.9071

07. Hien, D. A., et al. (2023). *Project Harmony: A meta-analysis with individual patient data on behavioral and pharmacologic trials for comorbid posttraumatic stress and alcohol or other drug use disorders*. American Journal of Psychiatry, 180(2), 155-166.

doi.org/10.1176/appi.ajp.22010071

RESEARCH AND REFERENCES

References 08-14

08. Cloitre, M., et al. (2010). *Treatment for PTSD related to childhood abuse: A randomized controlled trial*. American Journal of Psychiatry, 167(8), 915-924.

doi.org/10.1176/appi.ajp.2010.09081247

09. Cloitre, M., Courtois, C. A., Charuvastra, A., Carapezza, R., Stolbach, B. C., & Green, B. L. (2011). *Treatment of complex PTSD: Results of the ISTSS expert clinician survey on best practices*. Journal of Traumatic Stress, 24(6), 615-627.

doi.org/10.1002/jts.20697

10. van Vliet, N. I., et al. (2021). *Phase-based treatment versus immediate trauma-focused treatment for post-traumatic stress disorder due to childhood abuse: Randomised clinical trial*. BJPsych Open, 7(6), e211.

doi.org/10.1192/bjo.2021.1057

11. Mahoney, A., Karatzias, T., Halliday, K., & Dougal, N. (2020). *How important are Phase 1 interventions for complex interpersonal trauma? A pilot randomized control trial of a group psychoeducational intervention*. Clinical Psychology & Psychotherapy, 27(4), 597-610.

doi.org/10.1002/cpp.2447

12. Bhuptani, P. H., et al. (2024). *Interpersonal trauma, shame, and substance use: A systematic review*. Drug and Alcohol Dependence, 258, 111253.

doi.org/10.1016/j.drugalcdep.2024.111253

13. U.S. Department of Veterans Affairs, National Center for PTSD. *Overview of psychotherapy for PTSD*. Summary of the 2023 VA/DoD guideline. Consulted October 3, 2026.

www.ptsd.va.gov/professional/treat/txessentials/overview_therapy.asp

14. U.S. Department of Veterans Affairs, National Center for PTSD. *Treatment of co-occurring PTSD and substance use disorder in VA*. Clinical guidance summary. Consulted October 3, 2026.

www.ptsd.va.gov/professional/treat/cooccurring/tx_sud_va.asp

RESEARCH AND REFERENCES

References 15-20

15. Scott, C. K., Foss, M. A., & Dennis, M. L. (2005). *Pathways in the relapse-treatment-recovery cycle over 3 years*. Journal of Substance Abuse Treatment, 28(Suppl. 1), S63-S72.

doi.org/10.1016/j.jsat.2004.09.006

16. Khantzian, E. J. (1985). *The self-medication hypothesis of addictive disorders: Focus on heroin and cocaine dependence*. American Journal of Psychiatry, 142(11), 1259-1264.

pubmed.ncbi.nlm.nih.gov/3904487/

17. Joss, D., et al. (2026). *A randomized controlled trial of an online group-based internal family systems treatment for posttraumatic stress disorder: The Program for Alleviating and Resolving Trauma and Stress (PARTS) study*. Psychological Trauma: Theory, Research, Practice, and Policy. Advance online publication.

doi.org/10.1037/tra0002089

18. Ally, D., et al. (2025). *A pilot study of an online group-based Internal Family Systems intervention for comorbid posttraumatic stress disorder and substance use*. Frontiers in Psychiatry, 16, 1544435.

doi.org/10.3389/fpsy.2025.1544435

19. McLellan, A. T., Lewis, D. C., O'Brien, C. P., & Kleber, H. D. (2000). *Drug dependence, a chronic medical illness: Implications for treatment, insurance, and outcomes evaluation*. JAMA, 284(13), 1689-1695.

pubmed.ncbi.nlm.nih.gov/11015800/

20. Teicher, M. H., & Samson, J. A. (2016). *Annual research review: Enduring neurobiological effects of childhood abuse and neglect*. Journal of Child Psychology and Psychiatry, 57(3), 241-266.

pubmed.ncbi.nlm.nih.gov/26831814/